Bleeding in Early Pregnancy

Objectives: At the end of the lecture students will be able to:

- Define the term abortion.
- Classify the different types of abortion.
- Recognize the predisposing factors and causes of abortion.
- Differentiate between each type of abortion according to signs and symptoms.
- Define the term ectopic pregnancy.
- Diagnose ectopic pregnancy based on maternal symptoms.
- Define the term vesicular mole.
- Explain different approaches in diagnosis and management of vesicular mole.
- Provide care to mother has bleeding in early stage of pregnancy.

Outlines

- Definition of abortion.
- Types of abortion.
- Causes of abortion.
- Difference between types of abortion according to signs &symptoms
- Ectopic pregnancy.
- Vesicular mole.

Abortion

Definition of Abortion:

- It is a termination of pregnancy before viability of the fetus (A fetus of less than 20 weeks of gestation and weighing less than 350g is not considered viable
- Early abortion: 80% occurs before 12 weeks.
- Late abortion: is occurs between the 13th and 24th week of gestation.
- Ante partum Hge: bleeding that occurs after 28th week of gestation.
- Postpartum Hge: bleeding that occurs after delivery.

Causes of Abortion

- Genetic abnormalities.
- Tetratogenic agents as exposure to radiation, cytotoxic drugs, smoking, Infection as exposure to virus as rubella, cytomegalovirus, herpes simplex and toxoplasmosis.
- Placental abnormalities as uteroplacental ischemia.
- Endocrine deficiency as (progesterone deficiency, diabetes, poly cystic ovary syndrome, thyroid deficiency decreases TSH which leads to preterm labor).

- Congenital abnormalities of the uterus, cervical incompetence and immunological deficiencies.
- Physical and emotional trauma.
- Maternal age

Types of Abortion

1. Spontaneous:

- Threatened - Inevitable
- Complete - Incomplete
- Missed - Habitual

2. Induced

- Therapeutic. - Illegal (Criminal)

Threatened Abortion: It is one of subdivision of spontaneous abortion. It may go to term, or it may become inevitable.

Inevitable Abortion: This term is applied when it is impossible for the pregnancy to continue.

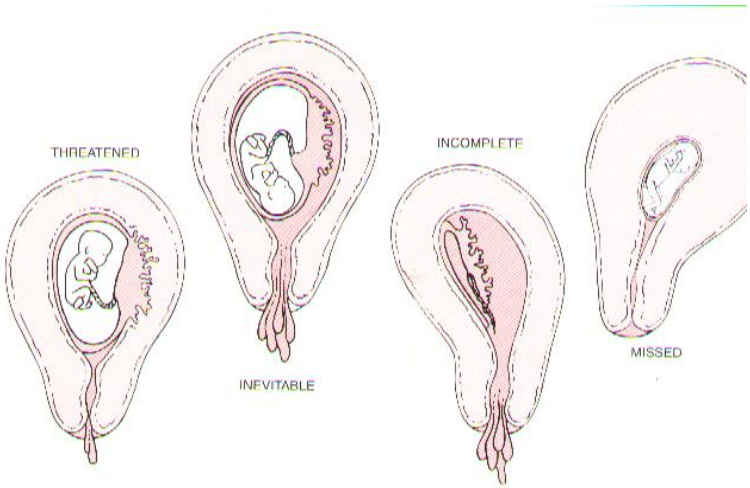
Incomplete Abortion: Some part of the products of conception has been expelled, while others (placenta & membrane) remain within the uterus.

Complete Abortion: All products of the conception are expelled.

Missed Abortion: Occurs when fetus dies and is not expelled but it is retained in uterus for two months or longer.

Septic Abortion: Incomplete abortion complicated by infection of the uterus cavity.

Habitual or Recurrent Abortion: Three or more consecutive spontaneous abortion.

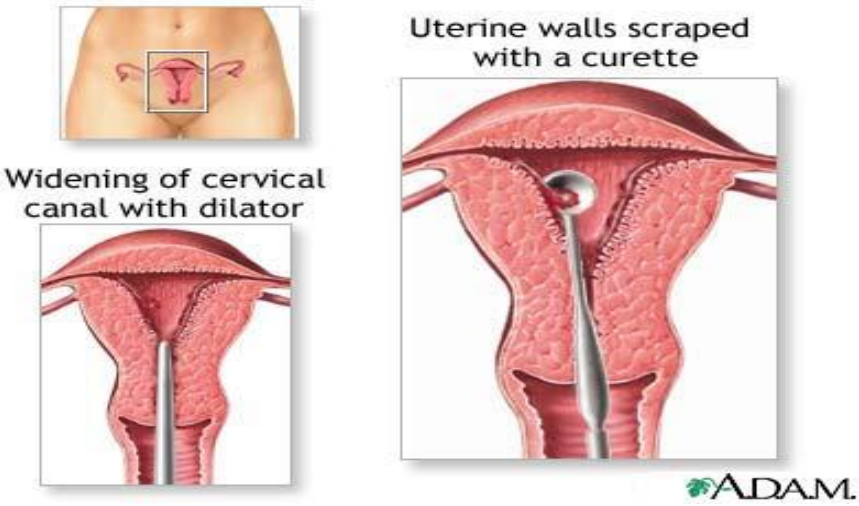


Types of Spontaneous Abortion and Usual Management

Type	Management
Threatened	Bed rest, sedation, and avoidance of stress and orgasm
Inevitable and Incomplete	Termination of pregnancy by dilatation and curettage (D&C)
Complete	If uterine contractions are adequate and if there is not infection no intervention may be needed
Septic	Immediate termination of pregnancy by method appropriate to duration of pregnancy. Cervical cultures and sensitivity

	studies and broad-spectrum antibiotics (e.g., ampicillin) is started. Treatment of septic shock is initiated if necessary.

Dilatation and Curettage (D&C)



Ectopic Pregnancy

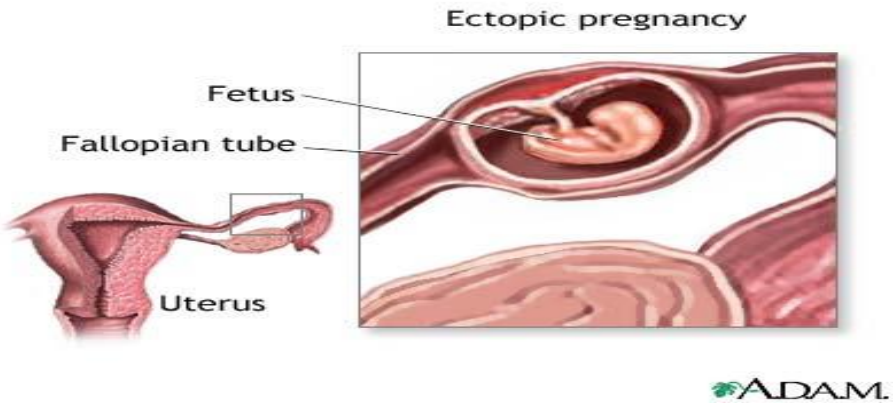
Definition:

- It is defined as the implantation of blast cyst somewhere other than in the endometrium of the uterus.
- It is a life-threatening condition.
- It accounts for approximately 10% of maternal deaths.

Sites of ectopic pregnancy

- The fallopian tube
- Ovary

- Cervix - Abdominal Cavity



Predisposing Factors:

- Pelvic inflammatory disease (PID)
- Previous ectopic pregnancy
- Congenital anomalies
- Simulation of ovulation
- Contraceptive failure IUD

Signs and symptoms

1-Before Rupture:

- Abdominal pain (dull – unilateral or bilateral).
- Amenorrhea (irregular period is reported in 90% of patients).

- Abnormal vaginal bleeding (mild to intermittent dark vaginal bleeding.
- Absence of common signs of pregnancy.
- Abdominal tenderness.
- Rupture (exacerbation of pain occurs during rupture in an ectopic pregnancy).

2. After rupture:

- Faintness (dizziness during bleeding).
- Generalized, unilateral abdominal pain (jumping sign).
- Referred shoulder pain caused or related to diaphragmatic irritation from
- Blood in the peritoneal cavity.
- Signs of shock.
- A febrile state.

Diagnosis:

* Beta subunit in blood * Ultrasound.

Treatment:

- Before rupture: Salpingostomy
- If rupture happened: Salpingectomy (removal of fallopian tube or salpingoopharectomy (removal of ovary and fallopian tube to control bleeding.

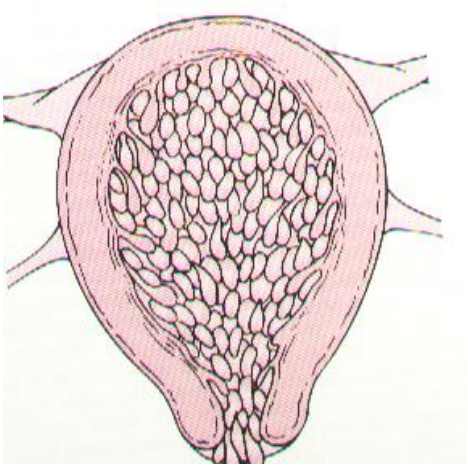
Hydatidcyst Mole (Vasculairmole)

Definition:

It is a benign proliferative growth of trophoblast in which chorionic villi develops into edematous, cystic transparent vesicles that hang in grape like clusters. It occurs in about 1 in every 1000 pregnancies.

Causes:

The causes are unknown, but it is theorized that an ovular defect, stress, nutritional deficiency especially protein.



Clinical Features

- Abnormal uterine bleeding which is intermittent and brown in color and passing of vesicles.
- Variable uterine size (uterus is larger than expected gestation).
- Ovarian enlargement due to elevated level of HCG
- Hyperemesis graviderum.
- Development of pre-eclampsia as protein in urine- elevated B.P and edema.

- Metastatic disease may present with pelvic pain, abnormal vaginal bleeding, or rectal bleeding- haemoptysis and dyspnea.

Diagnosis:

* Serial Beta – subunit for HCG *Ultrasound (snow -storm appearance).

Treatment

- Immediate evacuation by suction curettage followed by sharp curettage of the molar implantation site.
- Follow – up assessment:
- Chest x- ray
- Weekly or biweekly beta- subunit of HCG assay until the level drops to normal and remains normal for 3 successive weeks.
- Regular pelvic examination to assess uterine regression.
- Prophylactic chemotherapy.